

Diagnosis

- Balanitis and balanoposthitis are descriptive terms covering a variety of unrelated conditions, the appearances of which may be suggestive but should never be thought to be pathognomonic. Biopsy is sometimes needed to exclude pre-malignant disease.
- The following investigations are intended to aid diagnosis in cases of uncertainty:
 - Sexual history taken with specific questioning on sexual risk taking
 - Full routine screening for other sexually transmitted infections (STIs) including HIV as indicated by sexual history and presentation and in line with guidelines — for example:
 - HSV nucleic acid amplification test (NAAT) — if ulceration present
 - *Treponema pallidum* (TP) NAAT (or alternative test as per local availability) if an ulcer is present. Alternatively, check syphilis serology and repeat after 3 months
 - Screening for *Chlamydia trachomatis* infection / non-specific urethritis if a circinate-type balanitis is present
 - Subpreputial swab for *Candida* spp and bacterial culture — may be useful to exclude an infective cause or superinfection of a skin lesion or dermatosis
- Urinalysis for glucose — appropriate in some cases, especially if candidal infection is suspected
- Dermatology opinion for dermatoses and suspected allergy
 - Biopsy — if the diagnosis is uncertain and the condition is persistent

Management

The aims of management are to minimize sexual dysfunction, minimize urinary dysfunction, exclude penile cancer, treat pre-malignant disease, and diagnose and treat sexually transmitted diseases. Predisposing factors for balanoposthitis include poor hygiene, over-washing, non-retraction of the foreskin, and medical conditions such as diabetes mellitus.

Many cases of balanoposthitis seen in practice are a simple "intertrigo," that is, inflammation between two skin surfaces with bacterial or fungal overgrowth. Good personal hygiene, washing daily, avoiding irritants (such as soap), and keeping the foreskin retracted until the glans penis is dry (while advising the patient about the risk of paraphimosis, especially if the prepuce is tight) can be effective, but compliance may be challenging.

General advice

- Avoid soaps while inflammation is present
- Advise about risks of condom failure if creams are being applied to the glans or foreskin
- Patients should be given a detailed explanation of their condition, with particular emphasis on any implications for their health (and that of their partner, where a sexually transmissible agent is found)

Management of Specific Conditions

Infective balanoposthitis

A range of infective agents have been isolated more frequently in patients presenting with balanoposthitis and may not be easily differentiated by clinical findings. These include viral, bacterial, and fungal causes.